

# THE ARCHITECTURE RETURNS

*A Standalone Addendum to Consent Was Never the Point  
The Cross-Canada Institutional Rebuild, May 2026*

## **The Alberta Disability System Breakdown**

St. Albert, Alberta · May 2026

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## Executive Summary

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This addendum is a distillation of Chapters VI, VII, and VIII of *Consent Was Never the Point: Alberta's Eugenic Inheritance and Canada's Institutional Rebuild, 1928–Present*, the long-form documentation piece issued by The Alberta Disability System Breakdown in May 2026. It is published as a standalone document to support independent distribution to oversight bodies, civil-society organizations, and international human-rights mechanisms — in particular, the United Nations Special Rapporteur on the Rights of Persons with Disabilities.

**The thesis.** The institutional rebuild documented in the parent piece is not an Alberta-isolated development. As of May 2026, the same pattern is operationally installed in four Canadian provinces, has been the subject of formally announced planning in a fifth, and is the subject of explicit political demand — including a public demand for the use of the Constitution's notwithstanding clause — in a sixth. The framework crosses partisan lines: it is being pursued by United Conservative Party government in Alberta, Saskatchewan Party government in Saskatchewan, BC New Democratic Party government in British Columbia, and Manitoba New Democratic Party government in Manitoba. The continuity is institutional, not partisan.

**The architectural finding.** The infrastructure being designated for the contemporary framework's operation is, in multiple jurisdictions, the same infrastructure within which the historical eugenics framework operated. In Alberta, one of the five named retrofit facilities — the Centennial Centre, Ponoka — occupies the institutional grounds of Alberta Hospital Ponoka, identified in the standard academic reference as one of the four primary feeder hospitals of the Alberta Eugenics Board during 1929–1972. In Saskatchewan, the involuntary inpatient unit will operate at Saskatchewan Hospital North Battleford, a psychiatric institution in continuous operation since 1914. In British Columbia, both of the province's involuntary care facilities operate inside or on the grounds of correctional facilities.

**The targeted population.** Across all four operational jurisdictions, the named target population includes persons with combined mental-health, substance-use, and cognitive-impairment or brain-injury presentations. That intersection is the precise demographic position that the historical 1928–1972 framework targeted under the categories of "mentally defective" and "psychotic" persons. It is also the position of what this piece names the *missing middle* — the majority of the disabled population whose moderate disability requires case-by-case adjudication that contemporary disability supports are increasingly unable to provide.

**The CRPD compliance question.** The framework operates as substituted decision-making within the meaning of CRPD General Comment No. 1 (2014) on Article 12. The United Nations Committee on the Rights of Persons with Disabilities, in its Concluding Observations on Canada's second and third periodic reports adopted on March 18, 2025 (UN Doc CRPD/C/CAN/CO/2-3), identified continuing reliance on substituted decision-making as an outstanding compliance failure. The cross-Canada institutional rebuild documented in this addendum has, across all jurisdictions, expanded rather than reduced that framework.

*“The mechanism has changed. The architecture has not. The continuity is documentary, operational, budgetary, and continental.”*

## The Cross-Canada Operational Record

The following table summarizes, jurisdiction by jurisdiction, what has been legislated, what facilities have been opened or named for operation, and what populations have been identified as the framework’s target. Per-jurisdiction primary sources are cited in Chapter VIII of the parent piece.

| Jurisdiction                                           | Statutory mechanism                                                                                                                      | Operational footprint                                                                                                                                                                                                                                                                                                      | Status (May 2026)                                                                                                                 |
|--------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------|
| <b>Alberta</b><br>Gov: UCP<br>Premier Smith            | <i>Compassionate Intervention Act</i> , SA 2025, c C-21.5. Royal Assent May 15, 2025.                                                    | Five existing psychiatric facilities identified for retrofit (Alberta Hospital Edmonton; Centennial Centre Ponoka; Claresholm Mental Health Centre; Peter Lougheed Centre Calgary; Grande Prairie Regional Hospital) + Northern Alberta Youth Recovery Centre. Two new 150-bed centres (Edmonton, Calgary) scheduled 2030. | Commissioner appointed Aug 11, 2025. \$318.6M allocated Budget 2026. Regulations under development.                               |
| <b>Saskatchewan</b><br>Gov: Sask. Party<br>Premier Moe | <i>The Compassionate Intervention Act</i> . Royal Assent May 6, 2026.                                                                    | Assessment centre + involuntary inpatient unit named: Saskatchewan Hospital, North Battleford (psychiatric facility, operating since 1914).                                                                                                                                                                                | Force expected fall 2026. NDP’s 17 proposed amendments all rejected. Joint SMA/CPSS opposition statement Apr 1, 2026.             |
| <b>British Columbia</b><br>Gov: BC NDP<br>Premier Eby  | <i>Mental Health Act</i> , RSBC 1996, c 288 (expanded administrative use, no new primary legislation).                                   | Surrey Pretrial Services Centre involuntary care unit (April 2025, inside correctional facility). Alouette Homes, Maple Ridge (June 3, 2025, 18 beds, on grounds of Alouette Correctional Centre for Women).                                                                                                               | Operational. Eby framework introduced Sep 15, 2024. Health Minister Osborne confirmed: patients have no choice in treatment.      |
| <b>Manitoba</b><br>Gov: Manitoba NDP<br>Premier Kinew  | Protective care centre framework (72-hour involuntary detention for substance impairment).                                               | Winnipeg protective care centre, opened 2025, expanded March 4, 2026 to accept persons impaired by methamphetamine and other substances.                                                                                                                                                                                   | Operational and accepting non-alcohol-impaired persons as of March 2026.                                                          |
| <b>Ontario</b><br>Gov: PC<br>Premier Ford              | No new statute yet. <i>Mental Health Act</i> , RSO 1990, c M.7 and <i>Health Care Consent Act</i> , 1996 remain the governing framework. | Twelve Ontario mayors (Nov 7, 2024) demanded Premier Ford use s. 33 notwithstanding clause to enable mandatory drug/mental-health treatment. Brampton Mayor Brown publicly adopted the term “compassionate intervention.”                                                                                                  | Political demand active. Government studying BC model. Ontario Psychiatric Association running Mental Health Law Reform Campaign. |

| Jurisdiction                                         | Statutory mechanism                                                    | Operational footprint                                                                                         | Status (May 2026)                              |
|------------------------------------------------------|------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------|------------------------------------------------|
| <b>New Brunswick</b><br>Gov: Liberal<br>Premier Holt | Compassionate intervention legislation announced (modeled on Alberta). | No facility named as of May 2026. CBA—New Brunswick Branch has filed substantive legal opposition submission. | Planning announced. No legislation yet tabled. |

## The Architecture Returns: The Institutional-Continuity Finding

The framework documented in the parent piece is most often discussed in terms of its statutory provisions, its political sponsors, and its evidentiary justifications. This addendum centers the finding that is most directly relevant for human-rights monitoring purposes: where, physically, the framework is being installed.

**Alberta: the Ponoka link.** The Government of Alberta’s Budget 2026 allocates \$318.6 million over three years for the Compassionate Intervention Act’s implementation. Of that, \$27.7 million is allocated to retrofitting five existing publicly-funded hospital facilities to operate the framework before the dedicated centres open in 2030. One of the five named retrofit facilities is the Centennial Centre, Ponoka. The Centennial Centre occupies the institutional grounds of Alberta Hospital Ponoka. Alberta Hospital Ponoka is identified in Stahnisch and Kurbegović’s standard 2020 academic reference *Psychiatry and the Legacies of Eugenics* as one of the four primary feeder hospitals of the Alberta Eugenics Board during the period of the Sexual Sterilization Act’s operation, 1929 to 1972. The other three feeder hospitals identified in the academic record are Alberta Hospital Oliver (Edmonton), the Provincial Training School for Mental Defectives (Red Deer), and Deer Home (Red Deer). The institutional continuity between the historical Eugenics Board framework and the contemporary Compassionate Intervention Act framework is, in the case of Ponoka, a continuity of physical building, operational responsibility (the provincial mental-health agency), and provincial jurisdiction.

**The point is not motivational.** The point is not that the contemporary staff of the Centennial Centre have any connection to the historical framework, or that contemporary patients will be subjected to surgical sterilization. The point is that the institutional architecture the historical framework operated through has not been dismantled. It has been continuously occupied by successive frameworks. The Sexual Sterilization Act repealed in 1972 left the physical infrastructure intact. That infrastructure has now been formally designated, by the same provincial government that operated it during 1929–1972, as the operational platform for an involuntary-intervention framework targeting populations whose composition overlaps substantially with the populations the historical framework targeted.

**Saskatchewan and the psychiatric-hospital pattern.** Saskatchewan’s involuntary inpatient unit will operate at Saskatchewan Hospital North Battleford. Saskatchewan Hospital has operated as a psychiatric institution at North Battleford continuously since 1914. The first Compassionate Intervention Assessment Centre is also located at North Battleford. The Saskatchewan framework’s operational footprint, as in Alberta, sits inside the existing psychiatric hospital network.

**British Columbia and the correctional pattern.** British Columbia’s expansion of involuntary care has placed both of the province’s dedicated facilities inside or on the grounds of correctional facilities. The first BC facility — the involuntary care unit at the Surrey Pretrial Services Centre, opened April 2025 — is located inside a provincial correctional facility. The second BC facility — Alouette Homes, opened June 3, 2025 — is located on the grounds of the Alouette Correctional Centre for Women. Coverage by the Maple Ridge–Pitt Meadows News confirms the facility is “behind locked doors and within the gates of the correctional facility.”

## The Missing Middle: Who the Framework Targets

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The contemporary frameworks documented above identify their target population in similar terms across jurisdictions: persons with combined mental-health, substance-use, and cognitive-impairment or brain-injury presentations. That demographic intersection is the structural location this piece names the *missing middle*.

**The bimodal structure of disability supports.** Canadian disability supports are bimodally distributed. Intensive supports — group homes, residential placements, institutional care — are available to persons at the most severely disabled end of the spectrum. Minimal accommodations — workplace duty-to-accommodate, post-secondary disability services — are available at the most functional end. The substantial cohort between those two ends — persons with moderate intellectual disability, fluctuating mental illness, learning disabilities, executive-function disabilities, cognitive impairments that do not require twenty-four hour care but do require case-by-case adjustment to access employment, education, housing, and services — is the missing middle.

**The empirical proportion.** Statistics Canada’s most recent Canadian Survey on Disability (2022, released December 1, 2023) finds that fifty-nine percent of Canadians with disabilities have what Statistics Canada classifies as “milder” disabilities — mild or moderate. The missing middle is not a marginal sub-population. It is the majority of the disabled population.

**The employment record.** Statistics Canada’s 2017 employment data by severity shows employment rates of 76 percent for persons with mild disabilities, 67 percent for moderate, 49 percent for severe, and 31 percent for very severe. The 67 percent rate for the moderate cohort is the missing middle’s structural location: substantially below the general population (80 percent in the comparable period), substantially above the rates that justify intensive supports, and inadequately served by the accommodation-based supports available at the high-functioning end.

**The case-by-case adjudication problem.** What distinguishes the missing middle as a structural matter is the case-by-case variability of the supports required. The person whose functional capacity depends on a fluctuating mental-health presentation, whose access to a particular opportunity depends on a modification that is unlike the modification needed for the next opportunity, requires case-by-case adjudication by a person who knows their situation. Alberta’s Family Support for Children with Disabilities (FSCD) program supplied this adjudication through individual caseworkers; in September 2024, seven temporary FSCD caseworker contracts were not renewed, and the program’s direction of travel is toward online

intake, partial Family Support Services agreements without a caseworker, and three-year average waits for individualized caseworker assignment. The AISH-to-ADAP transition under Bill 12 is the adult-side analogue: eighty thousand AISH recipients, the substantial majority of whom present with cognitive, mental, or combined cognitive-and-mental conditions, are being moved into a program structured around employment-readiness rather than case-by-case adjudication, with appeal rights removed.

**The capture connection.** The population the institutional-rebuild framework most often names as its target is the population the structural disability framework has failed to serve on a case-by-case basis. The framework captures the population the structural framework has failed. The 1928–1972 Sexual Sterilization Act framework operated under the same structural logic, applying disproportionately to the population classified as “mentally defective” — the cohort the institutional framework of the period could not serve adequately on a case-by-case basis. The mechanism has changed. The structural position of the captured population has not.

## The CRPD Compliance Question

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Canada signed the Convention on the Rights of Persons with Disabilities on March 30, 2007 and ratified it on March 11, 2010. The Optional Protocol to the Convention was acceded to by Canada on December 3, 2018. As of May 2026, Canada is bound by both the Convention and the Optional Protocol.

**Substituted decision-making and Article 12.** CRPD General Comment No. 1 (2014) on Article 12 (UN Doc CRPD/C/GC/1, adopted 19 May 2014) establishes that substituted decision-making frameworks are incompatible with the Convention and must be replaced with supported decision-making regimes. The frameworks documented in this addendum operate as substituted decision-making in their explicit statutory design: the Alberta and Saskatchewan Compassionate Intervention Acts authorize binding treatment orders issued by an administrative commission on application by third parties; the BC Mental Health Act framework removes from the patient any choice in what treatments they receive; the Manitoba 72-hour facility detains without consent on the basis of substance impairment.

**Consultation and Article 4(3).** CRPD General Comment No. 7 (2018) on Article 4(3) and Article 33(3) (UN Doc CRPD/C/GC/7, 9 November 2018) establishes that persons with disabilities and their representative organizations must be closely consulted and actively involved in the development of laws and policies that affect them. The cross-Canada record documented in this addendum shows that consultation under the relevant legislative processes has been minimal in all jurisdictions: Alberta CIA passed without independent consultation with the disability community on the Act’s substantive provisions; Saskatchewan CIA passed with the government claiming consultation of “about 150 stakeholders”; British Columbia framework introduced via news release one week before the 2024 election campaign began.

**Concluding Observations on Canada (March 2025).** The Committee on the Rights of Persons with Disabilities adopted its Concluding Observations on the combined second and third periodic reports of Canada at its 779th meeting on March 18, 2025 (UN Doc CRPD/C/CAN/CO/2-3, document dated April 15, 2025). The Committee identified continuing reliance on substituted decision-making across Canadian

jurisdictions and the failure to implement supported decision-making consistent with Article 12 as outstanding compliance failures. The institutional rebuild documented in this addendum has, across the four operational and two planning Canadian jurisdictions, expanded rather than reduced the substituted decision-making framework the Committee identified as non-compliant. The expansion has occurred in the twelve months between the Committee's observations and the date of this addendum.

## Closing

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This addendum is a self-contained extract of the longer *Consent Was Never the Point*. Readers seeking the full evidentiary record — including the historical documentation of the 1928–1972 Sexual Sterilization Act period, the structural-eugenics analysis of contemporary Alberta disability policy, the AISH-to-ADAP transition under Bill 12, the information asymmetry analysis, and the full per-jurisdiction record — are referred to the parent piece, available at [\*\*albertadisabilitysystembreakdown.netlify.app\*\*](https://albertadisabilitysystembreakdown.netlify.app).

The Alberta Disability System Breakdown holds the work as collective record. No individual byline is asserted. The piece is free to share, free to print, free to post, under attribution to the campaign.

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### The Alberta Disability System Breakdown — Advocate

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